February 23, 2026

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Attention: HHS Health Sector AI RFI
Mary E. Switzer Building, 330 C Street SW
Washington, DC 20201

RE: Comments on Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care ("HHS Health Sector AI RFI")

EMI Advisors, LLC (EMI) is pleased to submit public comments to the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC), an agency within the Department of Health and Human Services (HHS). EMI is a nationally recognized health and human services interoperability firm with more than 15 years of experience supporting federal and state agencies in designing, governing, and implementing standards-based digital ecosystems.

EMI designs and guides interoperable systems, coordinated care models, and AI-ready digital infrastructure grounded in nationally recognized governance frameworks, including the National Institute of Standards and Technology (NIST) AI Risk Management Framework (AI RMF), the Coalition for Health AI (CHAI) Blueprint for Trustworthy AI, International Organization for Standardization/ International Electrotechnical Commission (ISO/IEC) 42001 Artificial Intelligence (AI) Management Systems standard, and ASTP/ONC Health Data, Technology, and Interoperability (HTI-1) requirements. We co-founded the Health Level Seven® (HL7) Gravity Project, the first HL7 Fast Healthcare Interoperability Resources® (FHIR) Accelerator focused on social determinants of health (SDOH), and authored multiple national FHIR implementation guides, including the electronic Long-Term Services and Supports (eLTSS) and Multiple Chronic Conditions (MCC) Care Plan specifications. These standards are now implemented or in production pilots across Centers for Medicare and Medicaid Services (CMS) programs, state Medicaid agencies, and provider networks.

Our recent work has focused on operationalizing trustworthy AI across the full lifecycle. This includes governed data foundations, documentation and assurance artifacts consistent with CHAI guidance, risk management practices aligned with NIST and ISO/IEC and conceptually aligned with the Food and Drug Administration (FDA) Good Machine Learning Practice (GMLP). EMI has articulated these themes in other public comments submitted to CMS, National Institutes of Health (NIH), Office of Science and Technology Policy (OSTP), and National Quality Forum (NQF). The recommendations below synthesize those positions and ground them in real-world implementation experience.

Responses

1. **What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?**

   The most persistent barrier is fragmented and low-fidelity data infrastructure. Many organizations operate in hybrid environments with inconsistent data standard implementation, limited semantic harmonization, and weak data provenance practices leading to models with lack of transparency and explainability. This lack of infrastructure undermines reproducibility, bias mitigation, safe scaling, and exacerbates existing resistance to AI innovation for health care.

Emerging assurance frameworks such as NIST AI RMF[1], Generative AI Profile[2], and the CHAI Blueprint for Trustworthy AI[3] all emphasize that high-quality, governed data and traceable inputs are foundational to trustworthy AI. Without standardized data products and lineage, model transparency and post-deployment monitoring are difficult to operationalize.

A second barrier is regulatory ambiguity. Health care organizations remain uncertain about how Health Insurance Portability and Accountability Act (HIPAA), 45 Code of Federal Regulations §164.514(b)[4], 42 CFR Part 2[5], and information blocking rules[6] (45 CFR Part 171)[7], apply to AI training, federated learning, synthetic data generation. This uncertainty delays deployment despite NIST's risk-based management approach articulated in federal guidance.

A third barrier is the lack of shared evaluation infrastructure. Safety-net providers and smaller organizations must independently validate AI tools without access to federated benchmarking environments, standardized test datasets, or cross-site evaluation protocols aligned with CHAI or NIST guidance. This duplication increases cost, reduces equity in access to innovation, and slows adoption.

EMI Perspective:
EMI's data modeling platform addresses these barriers by producing governed, standards-based schema models and standards mappings aligned to national FHIR implementation guides (e.g., Gravity SDOH, eLTSS, MCC Care Plan). The platform supports schema discovery, semantic harmonization, standards-aligned mapping artifacts, and conformance validation. These capabilities create a reusable, well-defined data foundation that enables reproducible AI evaluation, cross-site benchmarking, and infrastructure for safer post-deployment monitoring when AI analytics are applied downstream. The foundation provides support for CHAI and NIST aligned AI governance practices, reducing the resource burden that can be costly, duplicative, and inequitable for safety-net providers.

References:

- Innovaccer. (2026). State of Revenue Life cycle in Healthcare. https://content.innovaccer.app/hubfs/Flow/THE%20STATE%20OF%20RCM%20IN%20HEALTHCARE.pdf

[1] https://www.nist.gov/itl/ai-risk-management-framework
[2] https://www.nist.gov/publications/artificial-intelligence-risk-management-framework-generative-artificial-intelligence
[3] https://www.chai.org/workgroup/responsible-ai/blueprint-for-trustworthy-ai
[4] https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-C/part-164#164.514
[5] https://www.hhs.gov/hipaa/for-professionals/regulatory-initiatives/fact-sheet-42-cfr-part-2-final-rule/index.html
[6] https://healthit.gov/information-blocking/
[7] https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-D/part-171

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- Poon, E. G., Lemak, C. H., Rojas, J. C., Guptill, J., & Classen, D. (2025). Adoption of artificial intelligence in healthcare: survey of health system priorities, successes, and challenges. Journal of the American Medical Informatics Association: JAMIA, 32(7), 1093–1100. https://doi.org/10.1093/jamia/ocaf065

2. **What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.**

HHS should prioritize clarifying and aligning existing authorities rather than creating new AI-specific regulatory regimes.

a. Privacy and Data Use Clarification

HHS should issue guidance clarifying how HIPAA de-identification standards (45 CFR §164.514(b)) apply to AI training, synthetic data, and federated learning in a manner consistent with NIST AI RMF governance principles and ISO/IEC 42001 management system principles. Continued harmonization of 42 CFR Part 2 with HIPAA should clarify how de-identified or consent-governed data may be used for AI-driven care coordination and quality improvement, with strict safeguards against re-identification.

b. Payment Policy and Quality Measurement Modernization

CMS should use its existing authorities under:

- 42 CFR Parts 412 and 419 (Hospital IPPS/OPPS)[8],
- 42 CFR Part 414 (Medicare Physician Fee Schedule and Quality Payment Program)[9],
- 42 CFR Part 422 (Medicare Advantage)[10],
- 42 CFR Part 438 (§438.340 Medicaid quality strategies)[11],

to explicitly recognize and incentivize AI-enabled services that demonstrate outcomes, safety, and care coordination.

Modernization should include:

- Recognition of digitally derived quality measures,
- Use of AI-supported risk stratification within value-based programs,
- Acceptance of longitudinal, FHIR-based care planning artifacts from certified health IT as evidence of inputs.

[8] https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-B/part-419
[9] https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-B/part-414
[10] https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-B/part-422
[11] https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-C/part-438

Payment incentives should align with documented assurance artifacts consistent with CHAI, NIST AI RMF, and Joint Commission responsible AI guidance, such as model documentation, performance monitoring plans, and subgroup evaluation results.

c. ASTP/ONC Certification and Trusted Exchange Framework and Common Agreement™ (TEFCA) Alignment

ASTP/ONC should leverage authorities under 45 CFR Part 170 (Health IT Certification) and 45 CFR Part 171 (Information Blocking) to strengthen transparency and traceability requirements for predictive Decision Support Interventions (DSIs). HTI-1 creates the enforceable transparency lever for these interventions.

Certification criteria should require:

- Standardized data provenance and lineage,
- Traceable model input/output documentation,
- Audit logging of AI-influenced decisions, and
- Support for continuous performance monitoring.

As TEFCA matures, ASTP/ONC should clarify how Qualified Health Information Network (QHIN) participation may support AI-enabled use cases under Treatment, Payment, Health Care Operations, and Public Health exchange purposes, while maintaining compliance safeguards.

EMI Perspective:
EMI's platform enables certification-ready AI environments by producing governed schema models and standards-aligned mappings that support semantic harmonization and conformance validation. This creates the infrastructure required to support HTI-1 transparency expectations and TEFCA-enabled AI use cases.

3. **For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?**

Non-medical device AI introduces governance challenges that existing regulatory models were not designed to address. These include attribution of liability in multi-party ecosystems; management of model drift and performance degradation; accountability for continuously learning systems; automation bias and human oversight failures; and security risks, including prompt injection and data leakage in generative AI systems.

Existing governance models assume static software. AI systems are dynamic and socio-technical, with risk arising from workflows, data inputs, user interactions, and model behavior. HHS should convene stakeholders to harmonize expectations across agencies (ASTP/ONC, CMS, FDA, Agency for Healthcare Research and Quality (AHRQ), Office for Civil Rights (OCR)) and align with external frameworks such as CHAI, NIST AI RMF, FDA GMLP, and ISO/IEC 42001[12] AI management systems.

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[12] https://www.iso.org/standard/42001

Post-deployment monitoring and incident-reporting pathways should integrate with existing patient safety reporting structures, including with FDA Predetermined Change Control Plans.[13] AI-related adverse events should be incorporated into Patient Safety Organization reporting and learning networks. Shared liability frameworks should be linked to transparency, documentation, and auditability rather than algorithm design alone.

References:

- Elgin, C. Y., & Elgin, C. (2024). Ethical implications of AI-driven clinical decision support systems on healthcare resource allocation: a qualitative study of healthcare professionals' perspectives. BMC Medical Ethics, 25(1), 148. https://doi.org/10.1186/s12910-024-01151-8

4. **For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?**

Evaluation of AI in clinical care must be lifecycle-based and socio-technical. Accuracy testing alone is insufficient. Consistent with the NIST AI Risk Management Framework and CHAI's Blueprint for Trustworthy AI, evaluation should include clinical validity, clinical utility, reliability, subgroup performance, robustness under changing data conditions, workflow integration, and continuous monitoring for drift.

AI systems that support interoperability and multi-sector coordination require validation across heterogeneous environments rather than within a single organization. HHS should support the development of a federated testing infrastructure that allows participating health systems, Medicaid agencies, community-based organizations, and Qualified Health Information Networks to evaluate AI models locally using standardized evaluation protocols. Performance results can then be aggregated in de-identified form to enable benchmarking and shared learning without centralizing sensitive data.

A working model would include a standardized FHIR-based longitudinal data layer with harmonized terminology and embedded provenance metadata, common evaluation scripts aligned with CHAI and NIST guidance, and governance processes for monitoring performance over time. Such an infrastructure would also support structured change management processes conceptually aligned with FDA GMLP and predetermined change control planning principles.

HHS should also consider supporting regulatory sandbox environments that allow AI tools to be tested within defined governance and compliance boundaries prior to scaled deployment. These sandboxes can simulate exchange under TEFCA and evaluate interoperability across clinical, social, and long-term services and support settings. Digital twin environments or synthetic ecosystem simulations may further enable stress testing of AI models under variable policy, referral network, and data completeness conditions without exposing identifiable information.

[13] https://www.fda.gov/regulatory-information/search-fda-guidance-documents/predetermined-change-control-plans-medical-devices

EMI's work supporting the California Data Exchange Framework (DxF)[14] Community Sandbox[15] demonstrates how standards-aligned, FHIR-based exchange environments can be used to test multi-sector care coordination and referral workflows before live deployment. This model provides a scalable foundation for federated validation consistent with federal transparency and monitoring expectations.

Sustained cooperative agreements and public-private partnerships would be more impactful than isolated pilot grants. Durable testing infrastructure is essential for safe and sustainable AI adoption.

References:

- Sendak, M. P., D'Arcy, J., Kashyap, S., Gao, M., Nichols, M., Corey, K., Ratliff, W., Balu, S., & Barasch, E. (2020). A path for translation of machine learning products into healthcare delivery. NPJ Digital Medicine, 3, Article 59. https://doaj.org/article/7a4443d7de354f28b16c9f2934cb44e9

5. **How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?**

   HHS should avoid creating duplicative federal certification programs for AI. Instead, it should recognize and align with emerging consensus frameworks such as CHAI's Blueprint and Responsible AI Guide, the NIST AI Risk Management Framework, Joint Commission guidance on responsible AI use, and organizational governance standards such as ISO/IEC 42001.

   Federal programs can accelerate adoption by recognizing standardized assurance artifacts, including model documentation, data documentation, subgroup evaluation results, monitoring plans, and documented governance structures. Aligning expectations across ASTP/ONC, CMS, FDA, and NIH will reduce fragmentation and create a coherent signal to the private sector.

   HHS should also support open-source monitoring tools and shared evaluation resources so that smaller vendors and safety-net providers can participate in responsible AI adoption without incurring disproportionate costs. Capacity-building efforts consistent with data.org's governance and workforce development approaches would further strengthen implementation readiness across diverse communities.

6. **Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?**

   AI has met or exceeded expectations when deployed to reduce administrative burden, support risk stratification, and synthesize large volumes of structured data for population health management. It has fallen short when introduced into fragmented workflows without sufficient governance, change management, or monitoring.

[14] https://dxf.chhs.ca.gov/
[15] https://sbx.connectingforbetterhealth.com/

The literature demonstrates that performance gains often depend on integration into clinical processes rather than technical performance alone. Overestimation of return on investment and underinvestment in implementation science frequently undermine results.

The greatest opportunity lies in AI applications that synthesize longitudinal, multi-domain data to support care coordination, person-centered care planning, quality measurement, and integration of long-term services and supports and social care. These use cases augment clinical judgment and improve coordination rather than replace clinicians.

EMI's experience implementing national FHIR-based care planning and referral standards demonstrates that when standardized, computable longitudinal artifacts are available, AI use cases become more feasible, portable, and safe across organizations.

References:

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- Bates, D. W., Levine, D., Syrowatka, A., Kuznetsova, M., Craig, K. J. T., Rui, A., Jackson, G. P., Rhee, K., & Classen, D. C. (2021). The potential of artificial intelligence to improve patient safety: A scoping review. NPJ Digital Medicine, 4, Article 54. https://pubmed.ncbi.nlm.nih.gov/33742085/
- Eric G Poon, Christy Harris Lemak, Juan C Rojas, Janet Guptill, David Classen, Adoption of artificial intelligence in healthcare: survey of health system priorities, successes, and challenges, Journal of the American Medical Informatics Association, Volume 32, Issue 7, July 2025, Pages 1093–1100, https://doi.org/10.1093/jamia/ocaf065

7. **Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?**

Adoption decisions are typically driven by executive leadership, compliance and privacy officers, and information technology governance bodies. Clinical leaders play an important advisory role but often do not control procurement or enterprise risk decisions.

Administrative hurdles include regulatory uncertainty, lack of internal AI literacy, limited governance infrastructure, procurement constraints, and uncertainty regarding long term monitoring obligations. Organizations are particularly hesitant when accountability for model performance is unclear or when lifecycle oversight expectations are undefined.
HHS can mitigate these barriers by publishing clear implementation guidance aligned with CHAI and NIST frameworks, providing governance templates, supporting workforce training programs, and aligning payment incentives with documented assurance practices.

8. **Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.**

Enhanced interoperability would significantly widen market opportunities, fuel research, and accelerate responsible AI development. AI systems depend on consistent semantic normalization, longitudinal data, and large demographically diverse patient cohorts with reliable provenance to reduce the risk of bias and enhance accuracy.

Priority areas include FHIR-based care planning, such as the HL7 FHIR Multiple Chronic Conditions Care Plan Implementation Guide[16], HL7 FHIR electronic long-term services and supports artifacts, HL7 SDOH Clinical Care Implementation Guide[17], the CMS Post-Acute Care InterOperability (PACIO) Implementation Guides[18], and the HL7 FHIR Person-Centered Outcomes (PCO) Implementation Guide[19]. Adoption of these artifacts can further support transportable and reproducible AI evaluation across settings.

Standardized provenance metadata and conformance validation are equally important to ensure that AI inputs and outputs can be traced and audited across organizational boundaries.

EMI's data modeling platform operationalizes this approach by producing harmonized, FHIR aligned infrastructure that enables semantic harmonization and conformance validation. This infrastructure supports interoperable AI use cases and cross-site benchmarking consistent with federal certification and TEFCA expectations, providing the schema foundation on which longitudinal data, provenance, and cohort-based analytics can be built.

9. **What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?**

   Patients and caregivers seek AI that reduces repetitive data entry, improves navigation across systems, enhances care coordination, and respects individual preferences. They want AI to simplify complexity rather than introduce additional burden.

   Their primary concerns include inappropriate secondary use of data, lack of transparency regarding model training, bias and inequitable outcomes, and erosion of human oversight. Trust depends on meaningful disclosure of AI use, clear accountability pathways (e.g., HTI-1 DSI certification), and evidence that safeguards are in place.

   Consent-based architectures, equity monitoring, and transparent governance structures should be embedded from the outset. Responsible adoption must ensure that AI enhances patient agency rather than diminishing it.

10. **Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?**

    HHS should prioritize applied research that advances trustworthy implementation of AI in real-world clinical settings. This includes methods for bias detection and subgroup evaluation, drift monitoring, consent-aware system design, federated validation methodologies, and integration of clinical, social, and long-term services data.

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[16] https://build.fhir.org/ig/HL7/fhir-us-mcc/
[17] https://hl7.org/fhir/us/sdoh-clinicalcare/
[18] https://pacioproject.org/
[19] https://build.fhir.org/ig/HL7/pco-ig/en/

Research should extend beyond technical performance to assess workforce impact, redistribution of administrative burden, safety events, and sustainability over time. Implementation of science and organizational change research is particularly critical to understanding why some AI deployments succeed while others fail.

a. **Are there published findings about the impact of adopted AI tools and their use clinical care?**
b. **How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?**

Published findings show AI benefits are most consistent when it replaces manual aggregation and pattern recognition tasks within well-governed systems. Costs increase when AI is layered onto fragmented infrastructure, shifting work to care coordinators, information technology teams, and compliance staff. Longitudinal evaluation is necessary to assess whether burden reductions are durable and equitably distributed.

HHS can accelerate learning by supporting sustained evaluation networks and integrating AI monitoring into existing patient safety and quality reporting infrastructures.

Sincerely,

Evelyn Gallego

Evelyn Gallego, MBA, MPH, CPHIMS
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